Fax cover sheet

Lakeshore General Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Carla Mendes, RN CM
Phone: 555-0150
Date: 06/12/2026 08:45
Pages (including cover): 14
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

58M T2DM admitted for hyperglycemia, now controlled on oral agents.
Ready for d/c 06/12 PM. PT/OT, wound, SW notes and glucose log attached.
Please call CM with questions.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Lakeshore General Hospital (fictional) Dept/Floor: 6 East Medical
SW/CM/RN: Carla Mendes, RN CM Phone/Pager #: 555-0150
Email: cmendes@lakeshoregen.example Nursing Station #: 555-0151
Authorized by: Dr. N. Achterberg Phone #:

Patient Information

Patient Name: Daniel Doe MRN: LG-0572214 Date of Birth (dd/mm/yyy): 02/09/1968
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: White
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Hyperglycemia, uncontrolled T2DM
Admit Date: 06/08/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☒ Yes ☐ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 06/08/2026 Results: CXR no acute process

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes*	✕ No	Active Infection	Yes*	✕ No
*Explain			*Explain		
Self Administer All Meds	✕ Yes	No*	Ambulatory	✕ Yes	No
Continent of Bowel & Bladder	✕ Yes	No	Assistive Device	✕ Yes*	No
Colostomy/Ileostomy	Yes	✕ No	*Explain cane		
Foley Catheter	Yes	✕ No	Hospice	Yes*	✕ No
Diabetes	✕ Yes	No	*Explain		
Insulin	Yes	✕ No	Car	Yes	✕ No
Diabetes related Oral Meds	✕ Yes	No	Service Animal	Yes	✕ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ✕ No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain none

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Daniel Doe

DOB: 02/09/1968

Member ID: OCH-61842097

Referring Provider

Facility: Lakeshore General Hospital (fictional)

Clinical

Primary Diagnosis: Type 2 diabetes mellitus with hyperglycemia

ICD-10: E11.65

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 06/12/2026

Requested Days: 14

Secondary Dx: Type 2 DM with foot ulcer (E11.621); non-pressure chronic ulcer R foot, limited to breakdown of skin (L97.511)

Plan Use Only

Status: Pending review

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Patient face sheet

Lakeshore General Hospital (fictional) - PATIENT FACE SHEET

Patient: Daniel Doe
MRN: LG-0572214
Account #: 99572214
DOB: 02/09/1968
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 06/08/2026 19:05 (ED arrival 06/08/2026 14:20)
Admit Type: Inpatient
Unit/Bed: 6E-621A
Attending: Nora Achterberg, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date of Service: 06/08/2026

Attending: Nora Achterberg, MD (Hospital Medicine)

Chief Complaint

"My sugar has been over 400 on the shelter meter" x 1 week, polyuria, thirst

HPI

58 y/o M with T2DM (dx 2019) experiencing homelessness (sleeps at Example Shelter, Santa Ana, x 8 months) presents with 1 week of polyuria, polydipsia and fatigue after running out of metformin 3 weeks ago. Also reports a sore on the bottom of his right foot x 3 weeks, painless, no drainage, no fever. In ED: POC glucose 438, anion gap 11, serum beta-hydroxybutyrate 0.3 (negative). Not in DKA. No HHS (serum osm 296).

PMH

- T2DM, last A1c unknown prior to this admission
- Peripheral neuropathy, bilateral feet
- Hyperlipidemia

PSH

- None

Social History

Homeless x 8 months, staying at shelter. Former smoker (quit 2020). Denies alcohol or drug use.

Allergies

Sulfa (hives)

Home Medications

- Metformin 1000 mg PO BID (not taking x 3 weeks)
- Atorvastatin 20 mg PO daily

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History and Physical - page 2

Physical Exam

VS: T 98.9F, HR 96, BP 132/80, RR 16, SpO2 98% RA
Gen: NAD, AOx4
CV: RRR, no m/r/g
Lungs: CTAB
Skin/Feet: R plantar 1st metatarsal head ulcer approx 1.6 x 1.3 cm, shallow, clean base, no erythema, no warmth, no drainage, no odor. Probe-to-bone negative. Decreased monofilament sensation bilat. DP pulses 2+ bilat.
Psych: calm, cooperative, linear thought process

Assessment / Plan

58M with uncontrolled T2DM with hyperglycemia without DKA, and non-infected neuropathic R plantar foot ulcer.
1. Hyperglycemia - IVF, resume metformin, add sitagliptin; POC glucose AC/HS. No insulin planned unless persistent >300.
2. R foot ulcer, non-infected - X-ray R foot to r/o osteomyelitis; wound care consult; offloading post-op shoe; no antibiotics indicated
3. MRSA nares PCR per protocol
4. Dispo - CM consult; homeless, anticipate recuperative care referral
DVT ppx: enoxaparin 40 mg SC daily
Isolation: None - standard precautions
Code status: Full
Electronically signed: Nora Achterberg, MD 06/08/2026 21:30

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Wound care nurse note

WOUND CARE CONSULT NOTE

Date: 06/11/2026 10:20

Wound #1: R plantar foot, 1st metatarsal head, diabetic neuropathic ulcer, Wagner grade 1

Size: 1.4 cm L x 1.1 cm W x 0.2 cm D (improved from 1.6 x 1.3 on 06/09)

Wound bed: 100% pink granulation. Drainage: scant serous. Odor: none

Periwound: callus trimmed, no erythema, no induration, no signs of infection

Treatment

- Cleanse with NS, apply silver alginate, cover with bordered foam dressing
- Change dressing every other day and PRN; last changed 06/11/2026 AM
- Offloading: post-op shoe R foot at all times when walking

Education

Pt return-demonstrated dressing change independently x2 and daily foot self-inspection with mirror.

Recommendation

Home health RN wound check 3x/week (to match every-other-day dressing changes). Podiatry follow-up in 1-2 weeks.

Signed: G. Ilunga, RN CWOCN

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5

Date: 06/12/2026 07:30
S: Feels well, thirst resolved. Walking halls with cane. No foot pain.
O: T 98.2F HR 78 BP 124/76 RR 16 SpO2 98% RA
Glucose 118-152 over last 48h on metformin + sitagliptin. No hypoglycemia this admission. No insulin given this admission.
R foot X-ray 06/09: no osseous erosion, no osteomyelitis, no gas.
R foot ulcer improving per wound RN, no signs of infection.
A1c 9.6% (06/09).
A/P:
- T2DM with hyperglycemia, now controlled on oral agents. Continue metformin 1000 mg BID + sitagliptin 100 mg daily
- R plantar diabetic foot ulcer, non-infected, healing - dressing every other day, home health wound RN, podiatry f/u
- Medically stable for d/c to recuperative care today
Signed: Nora Achterberg, MD

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Case management and social work note

CASE MANAGEMENT / SOCIAL WORK NOTE

Date: 06/12/2026 09:10

Pt AOx4, pleasant and cooperative throughout stay, no behavioral concerns per nursing. Psychosocial screen: denies depression. Denies suicidal ideation. Denies homicidal ideation. No hallucinations reported or observed. No psychiatric history. No history of conservatorship or FSP.

Substance use: denies alcohol and drug use; no withdrawal signs.

Pt agreeable to recuperative care.

Arrangements

Home health referral accepted by Bayside Home Health (fictional), 555-0156; RN start of care visit scheduled 06/13/2026 for wound check and dressing change, then 3x/week.

Podiatry appointment: Lakeshore Podiatry Clinic (fictional) 06/22/2026 at 10:00 (confirmed).

PCP follow-up: Lakeshore Community Clinic (fictional) 06/18/2026 at 14:30 (confirmed) for diabetes.

Signed: Carla Mendes, RN CM

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Physical therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 06/10/2026 13:00

Dx: R plantar foot ulcer, peripheral neuropathy

Prior Level of Function

Independent ambulation, used a single-point cane outdoors for the past year.

Assessment

Bed mobility: Independent

Sit<>stand: Independent

Transfers: Independent

Gait: 150 ft with single-point cane in R post-op shoe, modified independent, no rest breaks, steady

Stairs: 6 steps with rail, modified independent

Balance: static good, dynamic good-

Plan / Recommendation

Safe for d/c to recuperative care with own cane and post-op shoe. Uses cane independently. No PT follow-up needs.

Signed: H. Varga, PT, DPT

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Occupational therapy evaluation

OCCUPATIONAL THERAPY EVALUATION

Date: 06/10/2026 15:15

ADLs

Grooming: Independent
UB dressing: Independent
LB dressing: Independent, dons post-op shoe independently
Toileting: Independent
Shower: Independent seated on shower bench, keeps R foot dressing dry with cast cover; gathers own supplies
Cognition: AOx4, follows multi-step commands, manages own meds with pill organizer

Recommendation

D/c to recuperative care. No further OT needs.
Signed: P. Okonkwo, OTR/L

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Glucose log and vital signs

POINT-OF-CARE GLUCOSE - LAST 5 CONSECUTIVE

Date/Time	Glucose (mg/dL)	Insulin given
06/10 21:00	214	none
06/11 07:00	176	none
06/11 11:30	152	none
06/11 21:00	141	none
06/12 07:00	118	none

No hypoglycemia this admission.

Pt checks own glucose with glucometer independently (teach-back 06/10, RN T. Lam).

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
06/11 08:00	98.4	82	128/78	16	98%	RA
06/11 12:00	98.6	80	126/78	16	97%	RA
06/11 20:00	98.3	76	122/74	16	98%	RA
06/12 00:00	98.0	74	120/72	14	98%	RA
06/12 07:30	98.2	78	124/76	16	98%	RA

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Laboratory results

LABORATORY RESULTS

Collected	Test	Result	Ref
06/08 14:40	Glucose	438 H	70-99 mg/dL
06/08 14:40	Anion gap	11	7-15
06/08 14:40	Beta-hydroxybutyrate	0.3	<0.6 mmol/L
06/08 14:40	WBC	8.2	4.0-11.0 K/uL
06/09 05:00	Hemoglobin A1c	9.6 H	<5.7 %
06/11 05:00	WBC	7.4	4.0-11.0 K/uL
06/11 05:00	Creatinine	0.9	0.7-1.3 mg/dL
06/11 05:00	CRP	0.6	<1.0 mg/dL

MICROBIOLOGY

06/09 06:00 MRSA nares PCR: NEGATIVE

06/12 06:15 SARS-CoV-2 rapid antigen: NEGATIVE

No wound culture sent (ulcer not clinically infected).

IMAGING

06/08 CXR PA/Lat: No acute cardiopulmonary process

06/09 R foot X-ray 3 views: No osteomyelitis. Soft tissue defect plantar 1st MT head.

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Discharge medication list

DISCHARGE MEDICATION RECONCILIATION

Patient: Daniel Doe MRN: LG-0572214

Continue / Resume

- Metformin 1000 mg PO twice daily with meals
- Atorvastatin 20 mg PO daily

New Medications

- Sitagliptin 100 mg PO daily
- Acetaminophen 650 mg PO every 6 hours PRN pain

Wound Supplies

- Silver alginate, bordered foam dressings (10 count), NS, cast cover

DME

- R post-op shoe (issued); own single-point cane

Stopped

- Enoxaparin (inpatient only)

No insulin at discharge. No antibiotics.

Patient able to self-administer all medications: Yes

30-day supply dispensed from outpatient pharmacy: Yes

Reviewed with patient by: S. Iyer, PharmD 06/12/2026

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.